

SUMMARY STATEMENT

PROGRAM CONTACT:
Crystal Henderson
crystal.henderson1@va.gov

(Privileged Communication)

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Application Number: 1I21RD002895-01

Principal Investigator

ZEVALLOS, JOSE P.

Applicant Organization: VETERANS HEALTH ADMINISTRATION

Review Group: HSR1
HSR-1 Health Care and Clinical Management

Meeting Date: 03/03/2026
Council: MAY 2026
Requested Start: 10/01/2026

Opportunity Number: RD-01-PILT
PCC: HSR-CH1

Project Title: Optimizing HPV Vaccination Strategies for Head and Neck Cancer Prevention in Veterans

SRG Action: Impact Score:172

Human Subjects: 30-Human subjects involved - Certified, no SRG concerns
Animal Subjects: 10-No live vertebrate animals involved for competing appl.
Participant Sex: 1A-Females and Males, scientifically acceptable
Participant Race and Ethnicity: 1A-Racial and/or Ethnic Minorities and Non-Racial and Ethnic Minorities, scientifically acceptable
Participant Age: 1A-Children, Adults, and Older Adults, scientifically acceptable
Clinical Research - not NIH-defined Phase III Trial

Project Year	Direct Costs Requested
1	115,000
2	115,000
TOTAL	230,000

ADMINISTRATIVE BUDGET NOTE: The budget shown is the requested budget and has not been adjusted to reflect any recommendations made by reviewers. If an award is planned, the costs will be calculated by VA Office of Research and Development (ORD) staff based on the recommendations outlined in the BUDGET COMMENT section and any relevant ORD service-specific limitations.

SUMMARY OF DISCUSSION:

Key Strengths (score driving):

- Addresses a critical knowledge gap in HPV vaccine underutilization among Veterans.
- The proposal is innovative as the first project to assess HPV vaccination practices and model the effects of expanded eligibility within the VHA.
- Findings have potential to inform meaningful practice or policy changes within the VA.
- The investigative team is highly qualified, with complementary expertise in HPV-related oropharyngeal cancer, microsimulation modeling, geospatial epidemiology, biostatistics, and VA data infrastructure.
- The approach applies a robust analytic framework that leverages the unique capabilities of VA and geospatial data linkage in a way that has not previously been done for this population.
- The project is supported by substantial preliminary work, including prior development of a microsimulation model that is ready to incorporate Veteran specific parameters, enhancing feasibility and applicability.
- Strong pathway to future Merit funding; the proposed analyses will generate data well suited to support a competitive Merit proposal.

Key Weaknesses (score driving):

- The proposal lacks sufficient detail on the burden of HPV-positive oropharyngeal cancer (OPC) in the VA compared to the general population (e.g., VA-specific incidence rates and trends).
- The impact of the project was not adequately justified due to uncertain low incidence of OPC and prior evidence showing little added benefit or cost-effectiveness of vaccination after age 26. There was some disagreement between reviewers about the underlying evidence supporting the incidence parameters used in their preliminary model estimates.
- The microsimulation model has some underspecified elements, including missing QALY (quality-adjusted life year) and cost-effectiveness analyses, insufficient rationale for a 10,000-patient simulation in the context of rare outcomes, and failure to adequately consider other adult vaccination history, age-specific vaccine adherence, pre-existing infection, and latency between infection and cancer diagnosis.
- The collection of Veteran-specific risk factors (e.g., military related exposures such as burn pits) are not clearly described, and these factors are not incorporated into the analytic framework.

DESCRIPTION (provided by applicant):

Significance to VA: Oropharyngeal cancer (OPC) caused by oncogenic human papillomavirus (HPV) is rapidly increasing in the United States and rising even faster among Veterans. While HPV vaccination is a proven, effective strategy for preventing persistent infection and HPV-related cancers, its uptake remains very low across the Veterans Health Administration (VHA), especially among mid-adult males, rural Veterans, and those entering VHA care after age 26. These gaps are particularly concerning because Veterans are more likely to be male, smoke, live in medically underserved areas, and face military-related exposures like burn pits, all of which could increase their risk for HPV-associated cancers. Notably, most HPV vaccination cost- effectiveness analyses have focused on cervical cancer and general-population assumptions that don't reflect Veterans' behavioral risks, healthcare usage patterns, or the later age at which many seek preventive care. This project addresses an urgent need to generate Veteran-specific evidence on HPV vaccination and to evaluate whether expanding vaccination eligibility through age 45 could lead to significant health and equity improvements. The work aligns closely with VHA priorities in cancer prevention, population health, and equitable access. Innovation and Impact: This study is the first to (1) comprehensively measure HPV vaccine uptake among Veterans aged 26–45 across rural and urban Veterans Integrated Service Networks (VISNs), and (2) develop a calibrated microsimulation model to project the long-term cancer prevention impact of expanding HPV vaccination eligibility within the VHA. It tackles a preventable cancer burden using VHA-specific data, considers military-related risk factors like smoking and burn pit exposure, and

incorporates real-world care patterns. The project's findings will enable data-driven, VISN-level prevention strategies and help close gaps in preventive care. Specific Aims: Aim 1. Determine the overall prevalence of HPV vaccination among Veterans aged 26-45 and measure current trends in HPV vaccination. Aim 2: Model the potential public health benefit of extending the age of universal HPV vaccination eligibility in the VHA population. Methodology: For Aim 1, we will build cohorts of Veterans aged 26–45 using data from 2018-2025 to estimate HPV vaccine uptake, series initiation, and adherence. Data will include demographics, behavioral risks, comorbidities, and access metrics to analyze disparities. Multivariable models will identify predictors of vaccination, and geographic mapping will highlight areas with low vaccination coverage. Aim 2 involves a microsimulation model that reflects the VHA population's characteristics, modeling transitions from HPV infection to OPC and to death. The simulation will evaluate lifetime outcomes under different policy scenarios, measuring disease burden and quality-adjusted life years gained. Sensitivity analyses will test assumptions and behavioral impact factors. Path to Translation/Implementation: Pilot findings will guide a future Veterans Affairs Merit Review on shared, co-designed HPV vaccination strategies in high-need VISNs. Based on Aim 1 results, we will identify key VISNs and work with Veterans, primary care teams, and leaders to develop tailored interventions, such as EHR decision support, pharmacist-led vaccines, nurse outreach, and reminder systems. The microsimulation model will project long-term impact and cost-effectiveness, informing VHA investments. Deliverables, including vaccination dashboards, simulation data, and risk maps, will be shared with relevant offices to support planning and policy development. This work creates a sustainable platform for Veteran-centered HPV prevention and sets the stage for broader implementation.

PUBLIC HEALTH RELEVANCE:

Veterans are experiencing rising rates of HPV-related throat cancer, a disease that currently has no screening test and is largely preventable through vaccination. This research will provide the first clear picture of how often Veterans receive the HPV vaccine, whether some Veteran groups face greater barriers to access, and how improving vaccination opportunities could reduce future cancer risk. By identifying gaps in care and estimating the potential benefits of expanded vaccination, this project aims to support more effective prevention strategies within the VA. The findings have the potential to improve long-term health, reduce cancer burden, and strengthen equitable access to preventive care for Veterans across the country. 1

CRITIQUE 1

Significance to VA:

Strengths:

- HPV-associated oropharyngeal cancer (OPC) is an epidemic, and its prevalence is disproportionately high among Veterans, establishing a strong and clinically urgent scientific premise with direct relevance to the VA health system.
- If successful, this project will provide actionable, VA-specific evidence to inform a policy of expanding HPV vaccination eligibility beyond age 26, representing a critical and potentially high-impact public health intervention for the Veteran population.
- The application aligns well with VA HSR priorities around Population Health and Whole Health by targeting HPV vaccination rates within the VHA, and with ORD-wide priorities by leveraging existing VA datasets and incorporating real-world economic modeling to guide vaccination policy.
- The study has generated stated interest from key VA stakeholders including the National Oncology Program Office, VISN Preventive Health Leads, the Office of Rural Health, and the Office of Health Equity, demonstrating organizational relevance and a meaningful path toward policy translation.
- The proposal includes a compelling future direction in the form of a Veteran Immunization Data Analytics (VIDA) platform to enable real-time monitoring of vaccine uptake and outcomes across

VISNs, and a clear tie-in to a subsequent Merit Review award involving an RCT and implementation-based analysis.

Weaknesses:

- It is unclear whether the VA is the optimal setting for studying this question exclusively, given the presence of several unique and potentially confounding risk factors in the Veteran population (including older age, higher rates of concurrent smoking, and distinct sexual behavior histories) that may complicate the assumptions underlying the microsimulation model and limit external generalizability.
- No VA-specific preliminary data on the incidence of HPV-associated OPC are presented, which makes it difficult to assess the magnitude of the disparity in this population and the potential absolute impact of expanded vaccination eligibility.
- The translational pathway from this largely descriptive and simulation-based study to discrete care delivery interventions is not well articulated. What is being modeled are primarily policy and guideline decisions rather than implementable clinical interventions, and it is unclear how findings will be operationalized within VA clinical workflows.
- The annual time steps used in the microsimulation model are coarse for modeling HPV natural history, given that infection clearance typically occurs within 12 to 24 months. This risks introducing discretization bias, and the application does not justify this choice or consider shorter time steps (e.g., quarterly) for the early infection states.

Innovation and Impact:

Strengths:

- This is the first study to comprehensively evaluate HPV vaccination practices among Veterans, including barriers to uptake, population-level trends, and the projected health impact of expanded vaccination eligibility within the VHA, representing a meaningful advance over existing literature that has not focused on this population.
- The use of a microsimulation model to evaluate the effects of different policy scenarios (expanding routine HPV vaccination eligibility to ages 30, 35, 40, or 45) is an innovative analytic approach for this research question in the VA context.
- The application links together unique VA data sources in a manner that could be leveraged beyond a cross-sectional retrospective project into a prospective monitoring platform, enhancing the long-term impact of the proposed infrastructure.
- The use of Spatial Access Ratios and the aggregation of individual-level and area-level variables is a novel and rigorous approach to characterizing vaccine uptake heterogeneity across geographically diverse Veteran populations.
- The combination of a cross-sectional study and a cohort design to address complementary research questions is a methodological strength that enhances the informativeness of the two aims.

Approach:

Strengths:

- Aim 1 uses a well-designed retrospective cross-sectional study to identify predictors of HPV vaccination among Veterans aged 26 to 45, with highly relevant predictor variables including primary care utilization and distance to care, and captures vaccination dose intensity, which may elucidate dose-response relationships.
- The Monte Carlo microsimulation model in Aim 2 is well-specified, with a five-state transition model (Healthy, Infection, Persistence, OPC, Death) that maps appropriately onto the understood natural history of HPV-associated oropharyngeal cancer, and transition probabilities between health states that are well-conceived.

- The VISN-level analyses to identify networks that may experience disproportionately large benefits from expanded vaccination eligibility provide a strong and actionable public health framing for the model outputs.

Weaknesses:

- The microsimulation model does not appear to capture several biologically important factors that could materially influence model outputs, including the HPV acquisition pathway (oral versus genital), the presence of existing infection at the time of vaccination, and type-specific vaccine efficacy, all of which may affect OPC incidence probability and natural history estimates.
- The latency period between persistent oral HPV infection and OPC diagnosis is long (likely 10 to 30 years), and the application does not describe how this latency is modeled. Specifically, it is unclear whether there is a dwell-time distribution or a fixed minimum duration in the Persistent Infection state, which is critically important for determining whether vaccinating Veterans at ages 40 to 45 would produce any meaningful cancer reduction within a 30-year projection window.
- The justification for simulating 10,000 patients rather than a larger cohort of 500,000 to 1,000,000 is not provided. Given the rarity of OPC as an outcome, a simulation of 10,000 individuals may be underpowered to produce stable estimates of cancer incidence reductions, and this choice requires explicit justification.
- It is unclear whether the transition probabilities used in the model are derived from published literature or are being actively measured in this study, and the uncertainty around these parameters is not addressed through sensitivity analyses or other methods of validation.

Feasibility:

Strengths: The team has a high-impact published study on HPV vaccination prevalence among the VA general medicine population aged 18 to 26, demonstrating the feasibility of extracting HPV vaccination data from the VA CDW (corporate data warehouse), identifying subgroups with lower vaccine uptake, and capturing vaccination status reliably in a VA-specific context. The proposed study extends this work to the larger 26-to-45 age group and adds the microsimulation component.

Weaknesses (minor): The feasibility of constructing and validating a five-state Monte Carlo microsimulation model within the timeframe and budget of a Pilot Award is not clearly established, particularly given the complexity of the latency and transition probability issues described above.

Investigators:

Strengths:

- Principal Investigator (PI) Dr. Zevallos is a head and neck surgeon and Chair of ENT at UPMC with longstanding collaborations with VA investigators and established expertise in the pathophysiology of HPV-related oropharyngeal cancer, making him a credible and well-positioned scientific leader for this work.
- The co-investigator team brings complementary and directly relevant expertise spanning decision science and microsimulation modeling (including CISNET experience), HPV-associated cancers, geospatial epidemiology, biostatistics, and the VA Corporate Data Warehouse (CDW).

VA Facilities and Resources:

Strengths: The environment, facilities, and resources are described as more than adequate to support the project, with access to the VA CDW and relevant informatics infrastructure through the co-investigator team.

Overall Strengths:

- The application addresses a highly significant and undercharacterized public health problem within the VA, as HPV-associated oropharyngeal cancer is disproportionately prevalent among

Veterans, and the study is the first to comprehensively evaluate HPV vaccination practices and model the projected impact of expanded eligibility within the VHA, with direct relevance to actionable vaccination policy change.

- The investigative team is well-suited for this work, with the PI bringing established expertise in HPV-related oropharyngeal cancer and a track record of VA collaboration, supported by co-investigators with directly complementary skills in microsimulation modeling with CISNET experience, geospatial epidemiology, biostatistics, and VA data infrastructure.
- The innovative combination of a retrospective cross-sectional cohort study using unique VA data sources with Spatial Access Ratios and area-level variables, linked to a Monte Carlo microsimulation model evaluating multiple vaccination eligibility expansion scenarios, provides a methodologically rich and policy-relevant analytic framework that leverages the unique capabilities of VA data in a way that has not previously been done for this population.

Overall Weaknesses: The microsimulation model has several critical underspecified elements that substantially limit confidence in its validity and interpretability, including the absence of QALY and cost-effectiveness modeling that would be expected for a policy-informing simulation, inadequate justification for the use of only 10,000 simulated patients for a rare outcome such as oropharyngeal cancer, failure to account for age-specific vaccine adherence rates, and unresolved questions about how HPV acquisition pathway, existing infection at the time of vaccination, and the long latency period between persistent infection and cancer diagnosis are captured within the five-state transition model.

Resubmission (if applicable): No comment.

Budget: No comment.

Sharing Research Data: No comment.

Biohazards: No comment.

Foreign/International Studies: No comment.

Select Agents: No comment.

Protection of Human Subjects: No comment.

Inclusion of Women, Minorities, and Children: No comment.

Vertebrate Animals: No comment.

CRITIQUE 2

Significance to VA:

- This proposal is highly significant to the VA. HPV vaccination rates among Veterans, particularly those aged 27-45 years, are alarmingly low. Increasing vaccination has clear potential to reduce HPV-associated oropharyngeal cancer (OPC) in this population.
- The findings are well positioned to inform a future Merit award, and the pathway to that next step is clearly articulated. The proposed analyses will provide valuable data to support development of a competitive Merit proposal.

Innovation and Impact:

- This proposal addresses a critical knowledge gap in understanding HPV vaccine underutilization among Veterans, a population with substantially lower vaccination rates than the general

population and disproportionately higher rates of HPV-associated oropharyngeal cancer (OPC). Generating VA-specific data to better characterize this disparity is both timely and necessary.

- The microsimulation model has already been developed and is ready to incorporate Veteran-specific parameters, enhancing its relevance and applicability to the VA population.
- In addition to informing development of a future Merit proposal, dissemination of findings from this study has the potential to directly support practice and policy changes within the VA.

Approach:

- The proposal does not sufficiently characterize the burden of HPV-positive OPC within the VA relative to the general population. To fully understand the potential impact of this work, the application should present VA-specific HPV-positive OPC incidence rates, trends over the past decade, and how both baseline rates and rate increases compare with non-VA populations.
- Clear documentation of the magnitude and trajectory of HPV-positive OPC among Veterans is essential for communicating the importance of this work. In reviewing the literature to better understand this burden, there were substantially different estimates of HPV-positive OPC within the VA (e.g., from the investigators' prior work vs another study reporting VA-based estimates). The team's prior study appears to use VA data but includes no VA investigators and all the authors are from industry and a for-profit research organization, making it unclear how the underlying data were accessed and analyzed. Presenting these estimates in the proposal would have facilitated evaluation of HPV-positive OPC burden in the VA population.
- Figure 4 identifies MSM (men who have sex with men) status as a risk factor; however, it is unclear whether this variable can be reliably captured in the VA data. Clarification is needed regarding how this exposure will be measured.
- Figure 4 would benefit from inclusion of Veteran-specific risk factors, such as military-related exposures (e.g., burn pit exposure). The proposal should clearly describe how these exposures will be identified and captured within VA data sources.
- Clear justification and rationale for the incremental age thresholds (up to 30, 35, 40, and 45) should be provided. The proposal should also consider the practical implications of recommending routine vaccination for some age groups within the 26-45 age range (e.g. up to age 35 based on the findings) and continued recommendations for shared decision-making among others (e.g. 36-45), as this mixed approach could create confusion for both patients and providers.
- Collection of and adjustment for other adult vaccination history should be included, as vaccination practices are often interrelated and may influence HPV vaccine uptake.

Feasibility: No major feasibility concerns were identified. The team demonstrates strong familiarity with VA data sources and has prior experience with the data and microsimulation model, supporting successful execution of the proposed work.

Investigators: The study team is appropriately qualified to conduct the study proposed.

VA Facilities and Resources:

- Facilities and resources appear appropriate.
- The application included evidence of support from the applicant's VA facility.

Overall Strengths:

- Highly significant to the VA. HPV vaccination rates among Veterans, particularly ages 27-45, are alarmingly low, and increasing vaccination has clear potential to reduce HPV-associated oropharyngeal cancer (OPC).
- Addresses a critical knowledge gap in HPV vaccine underutilization among Veterans, a population with both lower vaccination rates and higher OPC burden compared to the general population.

- The microsimulation model has already been developed and is ready to incorporate Veteran-specific parameters, enhancing feasibility and applicability.
- Strong pathway to future Merit funding; the proposed analyses will generate data well suited to support a competitive Merit proposal.
- Findings have potential to inform meaningful practice or policy changes within the VA.

Overall Weaknesses:

- The proposal does not sufficiently characterize the burden of HPV-positive OPC within the VA relative to the general population. VA-specific incidence rates and time trend, along with comparison to non-VA populations, are needed to fully justify the projected impact and policy relevance.
- Veteran-specific risk factors (e.g., military-related exposures such as burn pits) are not clearly incorporated into the analytic framework, and the proposal does not describe how these exposures would be captured.
- The rationale for modeling incremental age-based vaccination thresholds is unclear. From a policy standpoint, recommending routine vaccination through age 30 or 35 while maintaining shared decision-making for ages 31-45 or 36-45 may create implementation complexity and confusion.
- Adjustment for other adult vaccination history is not described and will be important to include, as vaccination behaviors are interrelated.

Resubmission (if applicable): Not applicable.

Budget: The budget is appropriate.

Sharing Research Data: The DMAP is sufficient.

Biohazards: Not applicable.

Foreign/International Studies: Not applicable.

Select Agents: Not applicable.

Protection of Human Subjects: Human subjects protections are sufficient.

Inclusion of Women, Minorities, and Children:

- Participants include women and individuals from underrepresented groups.
- Inclusion of children is not applicable.

Vertebrate Animals: Not applicable.

CRITIQUE 3

Significance to VA: This proposal addresses a relevant policy issue given rising HPV-associated oropharyngeal cancer (OPC) among Veterans. The significance is strengthened by documented disparities in vaccination rates within the VHA and evidence supporting the disproportionate OPC incidence among Veterans. Ultimately, significance will depend on whether it can demonstrate that Veteran-specific factors meaningfully modify prior conclusions regarding vaccination effectiveness in older adults rather than replicating them in a different population. This is appropriate for a pilot project.

Innovation and Impact: Innovation revolves around the use of VHA data and inclusion of military-related risk factors to determine barriers to HPV vaccination and calibrate established models to assess the benefits of vaccination policies on HPV-related cancer outcomes within VA-specific considerations.

Approach: The goal of this project is to first characterize HPV vaccination behavior among Veterans aged 26–45 and identify predictors of uptake, and second to use these data to calibrate a microsimulation model assessing the projected benefit of extending HPV vaccination eligibility beyond age 26. The approach is supported by strong preliminary work demonstrating feasibility in quantifying vaccination prevalence within the VHA and simulating population-level health and economic outcomes under alternative policy scenarios. Overall, the approach is methodologically rigorous. In Aim 1, vaccination outcomes are carefully defined, including consideration of non-VA vaccination, and predictors are thoughtfully constructed using validated VA phenotyping tools and datasets, which strengthens internal consistency and reproducibility. The statistical methods are appropriate and well described. Aim 2 benefits from an already developed microsimulation framework and explicit plans to calibrate the model to achieve alignment with Veteran-specific data from Aim 1. Although Aim 2 success depends on Aim 1, planned sensitivity analyses reduce this concern. However, important validity questions remain. The model will rely on RCT (randomized controlled trial)-derived vaccine efficacy estimates, yet Veterans may have a higher baseline prevalence of prior HPV exposure than the general population. It is unclear how well general-population efficacy estimates translate to mid-adult Veterans. On this subject, the proposal specifically discusses sensitivity analyses and calibrations that are rather unclear as to what simulation outputs can be compared to what observable Veteran data from Aim 1 given that Aim 1 does not measure HPV infection or HPV-related OPC. In the sensitivity analyses as planned, it is unclear which empirical benchmarks will be used for calibration. A strength of the approach is the inclusion of behavioral sensitivity scenarios, recognizing that provider practices and Veteran attitudes influence uptake, which may provide valuable information to plan a future Merit award proposal. Overall, while the analytic framework is sophisticated and calibrated to best available Veteran data, the validity of long-term projections will depend on assumptions, and the project's utility will thus depend to the extent to which simulated outcomes can be meaningfully validated against observable data is unclear.

Feasibility: The project is highly feasible, as prior work has already demonstrated the ability to extract and analyze HPV vaccination data from the VHA CDW and identify uptake patterns, and to develop a calibrated microsimulation model showing projected reductions in OPC cases, deaths, and costs under expanded vaccination scenarios, providing a strong methodological and operational foundation for the proposed aims.

Investigators: Experienced, productive PI with expertise in HPV-related OPC and supported by a team with complementary expertise needed including simulation modeling, head and neck cancer epidemiology and biostatistics.

VA Facilities and Resources: The facilities and environment at the Center for Healthcare Evaluation, Research, and Promotion (CHERP) are well-suited for the research.

Overall Strengths: The proposal's main strengths are its rigorous, data-driven approach applied to the timely issue of rising OPC incidence and its strong feasibility, supported by substantial preliminary work demonstrating the ability to extract and analyze VHA vaccination data and integrate these findings into simulation models that project clinically meaningful outcomes to inform policy decisions.

Overall Weaknesses: Overall impact may be moderated by the relatively low absolute incidence of OPC and prior evidence suggesting limited incremental benefit and cost-effectiveness of vaccination beyond age 26. It is unclear if there is any opportunity for that balance to change even with higher incidences and non-ideal screening scenarios. Thus, while the project could provide important Veteran-

specific modeling data to inform VHA policy, its potential to alter Veterans population-level outcomes may be constrained.

Resubmission (if applicable): No comment.

Budget: The budget is appropriate.

Sharing Research Data: DMAP is appropriate; risks of loss of confidentiality with large data outweigh sharing concerns.

Biohazards: Not applicable.

Foreign/International Studies: Not applicable.

Select Agents: Not applicable.

Protection of Human Subjects: Protections are adequate for this retrospective records review study.

Inclusion of Women, Minorities, and Children: There are adequate plans for inclusion of women and minorities.

Vertebrate Animals: Not applicable.

MEETING ROSTER

HSR-1 Health Care and Clinical Management

Health Services Research Parent IRG

Office of Research & Development

HSR1

03/03/2026 - 03/04/2026

CHAIRPERSON(S)

KOWALKOWSKI, MARC, PHD
ASSOCIATE PROFESSOR AND ASSOCIATE CHIEF FOR
RESEARCH, SECTION OF HOSPITAL MEDICINE
ASSOCIATE DIRECTOR, CENTER FOR HEALTH SYSTEM
SCIENCES WAKE FOREST UNIVERSITY SCHOOL OF
MEDICINE
ATRIUM HEALTH
WINSTON-SALEM, NC 27101

TAVEIRA, TRACEY H, PHMD
HEALTH RESEARCH SCIENTIST/CLINICAL PHARMACY
SPECIALIST
PROVIDENCE VA MEDICAL CENTER
PROFESSOR
WARREN ALPERT SCHOOL OF MEDICINE, BROWN
UNIVERSITY
UNIVERSITY OF RHODE ISLAND COLLEGE OF PHARMACY
PROVIDENCE, RI 02908

MEMBERS

ANDRAKA-CHRISTOU, BARBARA, PHD
ASSISTANT PROFESSOR
UNIVERSITY OF CENTRAL FLORIDA
ORLANDO, FL 32828

BENTREM, DAVID J, MD
STAFF PHYSICIAN
CHICAGO VA HEALTHCARE SYSTEM
PROFESSOR OF SURGERY
NORTHWESTERN UNIVERSITY
CHICAGO, IL 60612

BENZER, JUSTIN K, PHD
PROFESSOR
UTHEALTH HOUSTON SCHOOL OF PUBLIC HEALTH
AUSTIN, TX 78701

BILCHICK, KENNETH C, MD
PROFESSOR OF MEDICINE
DIVISION OF CARDIOVASCULAR
DEPARTMENT OF MEDICINE
UNIVERSITY OF VIRGINIA
CHARLOTTESVILLE, VA 22908

BRADY, BRIAN M, MD
CLINICAL ASSOCIATE PROFESSOR OF MEDICINE
STANFORD UNIVERSITY SCHOOL OF MEDICINE
STANFORD, CA 94305

BUHR, RUSSELL G, MD, PHD
STAFF PHYSICIAN AND CORE INVESTIGATOR
WEST LOS ANGELES VA MEDICAL CENTER
ASSISTANT PROFESSOR OF MEDICINE AND HEALTH POLICY
UNIVERSITY OF CALIFORNIA, LOS ANGELES
LOS ANGELES, CA 90024

BULLS, HAILEY W, PHD
ASSISTANT PROFESSOR
DEPARTMENT OF ANESTHESIOLOGY AND
PERIOPERATIVE MEDICINE
UNIVERSITY OF ALABAMA AT BIRMINGHAM
BIRMINGHAM, AL 35233

BUTLER, JORIE M, PHD
GRECC; CORE INVESTIGATOR COIN
SALT LAKE CITY VA MEDICAL CENTER
ASSISTANT PROFESSOR DIVISION OF GERIATRICS
DEPT. OF INTERNAL MEDICINE DEPT. OF BIOMED
INFORMATICS UNIVERSITY OF UTAH SCHOOL OF MEDICINE
SALT LAKE CITY, UT 84148

CAFFREY, AISLING R, PHD
CHAIR OF THE INSTITUTIONAL REVIEW BOARD AND
RESEARCH HEALTH SCIENTIST
PROVIDENCE VA MEDICAL CENTER
PROFESSOR OF HEALTH OUTCOMES
UNIVERSITY OF RHODE ISLAND
KINGSTON, RI 02881

CHHATRE, SUMEDHA, PHD
RESEARCH HEALTH SCIENTIST
PHILADELPHIA VA MEDICAL CENTER
RESEARCH ASSOCIATE PROFESSOR
PERELMAN SCHOOL OF MEDICINE
UNIVERSITY OF PENNSYLVANIA
PHILADELPHIA, PA 19104

CLOUTIER, RENEE M, PHD
ASSISTANT PROFESSOR
UNIVERSITY OF PITTSBURGH SCHOOL OF MEDICINE
PITTSBURGH, PA 15260

DAMUSH, TERESA M, PHD
SENIOR RESEARCH CAREER SCIENTIST
INDIANA VA HEALTHCARE SYSTEM
PROFESSOR OF GENERAL INTERNAL MEDICINE
INDIANA UNIVERSITY SCHOOL OF MEDICINE
INDIANAPOLIS, IN 46202

DOLL, JACOB A, MD
STAFF CARDIOLOGIST, DEPUTY SECTION HEAD
VA PUGET SOUND HEALTH CARE SYSTEM
ASSOCIATE PROFESSOR OF MEDICINE
UNIVERSITY OF WASHINGTON
SEATTLE, WA 98195

ELLERBECK, EDWARD F, MD
PROFESSOR
DEPARTMENT OF POPULATION HEALTH
UNIVERSITY OF KANSAS MEDICAL CENTER
KANSAS CITY, KS 66160

ELLIS, SHELLIE D, PHD
PROFESSOR
UNIVERSITY OF WISCONSIN, MADISON
MADISON, WI 53705

FIX, GEMMAE M, PHD
ACTING ASSOCIATE DIRECTOR
CENTER FOR HEALTH OPTIMIZATION & IMPLEMENTATION
RESEARCH VA BEDFORD HEALTHCARE SYSTEM
ASSOCIATE PROFESSOR
BOSTON UNIVERSITY SCHOOL OF MEDICINE
BEDFORD, MA 01730

GEBREGZIABHER, MULUGETA G, PHD
HEALTH RESEARCH SCIENTIST
CHARLESTON VA MEDICAL CENTER
PROFESSOR OF BIOSTATISTICS
DEPARTMENT OF PUBLIC HEALTH SCIENCES
MEDICAL UNIVERSITY OF SOUTH CAROLINA
CHARLESTON, SC 29425

GIFFORD, ALLEN L, MD
DIRECTOR, CENTER OF INNOVATION
VA BOSTON HSR&D CENTER OF EXCELLENCE
PROFESSOR OF PUBLIC HEALTH AND MEDICINE
BOSTON UNIVERSITY
BOSTON, MA 01730

GOLDBERG, DEBORA G, PHD
ASSOCIATE PROFESSOR
GEORGE MASON UNIVERSITY
FAIRFAX, VA 22030

GRIAUZDE, DINA H, MD
STAFF PHYSICIAN
ANN ARBOR VA MEDICAL CENTER
ASSISTANT PROFESSOR
UNIVERSITY OF MICHIGAN
ANN ARBOR, MI 48105

JACKSON, GEORGE L, PHD
RESEARCH HEALTH SCIENCE SPECIALIST
DURHAM VA MEDICAL CENTER
PROFESSOR
UNIVERSITY OF TEXAS SOUTHWESTERN MEDICAL CENTER
DURHAM, NC 27705

KRISHNAN, JERRY A, PHD, MD
PROFESSOR AND ASSOCIATE VICE CHANCELLOR FOR
POPULATION HEALTH SCIENCES
OFFICE OF THE VICE PRESIDENT FOR HEALTH AFFAIRS
UNIVERSITY OF ILLINOIS CHICAGO
CHICAGO, IL 60608

LOWENSTEIN, LISA M, PHD
ASSOCIATE PROFESSOR
DIVISION OF CANCER PREVENTION
AND POPULATION SCIENCES
DEPARTMENT OF HEALTH SERVICES RESEARCH
THE UNIVERSITY OF TEXAS MD ANDERSON CANCER
CENTER
HOUSTON, TX 77230

MITSOURAS, DIMITRIOS, PHD
PHYSICIST
SAN FRANCISCO VA HEALTHCARE SYSTEM
PROFESSOR IN RESIDENCE
DEPARTMENT OF RADIOLOGY AND BIOMEDICAL IMAGING
UNIVERSITY OF CALIFORNIA SAN FRANCISCO
SAN FRANCISCO, CA 94121

MOHANTY, APRIL F, PHD
RESEARCH HEALTH SCIENTIST
SALT LAKE CITY VA HEALTHCARE SYSTEM
RESEARCH ASSOCIATE PROFESSOR
UNIVERSITY OF UTAH- SPENCER FOX ECCLES
SCHOOL OF MEDICINE
SALT LAKE CITY, UT 84132

PAINTER, JACOB T, PHD
CO-DIRECTOR, CENTER FOR MENTAL HEALTHCARE AND
OUTCOMES RESEARCH
LITTLE ROCK VA MEDICAL CENTER
ASSOCIATE PROFESSOR
UNIVERSITY OF ARKANSAS FOR MEDICAL SCIENCES
LITTLE ROCK, AR 72227

PARIKH, RAVI B, MD
ASSOCIATE PROFESSOR
ATLANTA VA MEDICAL CENTER
ASSOCIATE PROFESSOR
EMORY UNIVERSITY
ATLANTA, GA 30319

RAPOPORT, SARAH K, MD
ATTENDING PHYSICIAN
WASHINGTON VA MEDICAL CENTER
ASSISTANT PROFESSOR
GEORGETOWN UNIVERSITY
WASHINGTON, DC 20422

ROOSAN, MOOM R, PHD
ASSOCIATE PROFESSOR
PHARMACY PRACTICE DEPARTMENT
CHAPMAN UNIVERSITY SCHOOL OF PHARMACY
IRVINE, CA 92618

ROUMIE, CHRISTIANNE L, MD
DIRECTOR, VETWISE LHS COIN, STAFF PHYSICIAN
NASHVILLE VA MEDICAL CENTER
PROFESSOR
DEPARTMENT OF INTERNAL MEDICINE AND PEDIATRICS
VANDERBILT UNIVERSITY
NASHVILLE, TN 37212

RUSTAGI, ALISON S, MD, PHD
STAFF PHYSICIAN
SAN FRANCISCO VA MEDICAL CENTER
ASSISTANT PROFESSOR
UNIVERSITY OF CALIFORNIA SAN FRANCISCO
SAN FRANCISCO, CA 94121

SHUKLA, ALPANA, MD
ASSOCIATE PROFESSOR OF RESEARCH
WEILL CORNELL MEDICINE
NEW YORK, NY 10021

SLATORE, CHRISTOPHER G, MD
PHYSICIAN
PORTLAND VA HEALTHCARE SYSTEM
PROFESSOR
DIVISION OF PULMONARY AND CRITICAL CARE MEDICINE
OREGON HEALTH AND SCIENCE UNIVERSITY
PORTLAND, OR 97239

SPANAKIS, ILIAS, MD
CHIEF, ENDOCRINOLOGY, DIABETES AND NUTRITION
SECTION
DIVISION OF ENDOCRINOLOGY, DIABETES AND NUTRITION
BALTIMORE VA HEALTHCARE SYSTEM
ASSOCIATE PROFESSOR OF MEDICINE
UNIVERSITY OF MARYLAND
BALTIMORE, MD 21201

SUNDAR, KRISHNA M, MD
STAFF PHYSICIAN
SALT LAKE CITY VA HEALTHCARE SYSTEM
CLINICAL PROFESSOR AND MEDICAL DIRECTOR
SLEEP AND PULMONARY SECTION, DEPT OF MEDICINE
UNIVERSITY OF CALIFORNIA, DAVIS
SACRAMENTO, CA 95817

VAVRA, ASHLEY K, MD
STAFF VASCULAR SURGEON AND CHIEF MEDICAL
DIRECTOR
CHICAGO VA HEALTH CARE SYSTEM
ASSOCIATE PROFESSOR
DEPT. OF SURGERY, DIVISION OF VASCULAR SURGERY
NORTHWESTERN UNIVERSITY
CHICAGO, IL 60622

WANG, YUN, PHD
ASSISTANT PROFESSOR
SCHOOL OF PHARMACY
CHAPMAN UNIVERSITY
IRVINE, CA 92618

WARD, RALPH C JR, PHD
RESEARCH SCIENTIST (BIostatistician)
CHARLESTON VA HEALTHCARE SYSTEM
RESEARCH ASSOCIATE PROFESSOR
MEDICAL UNIVERSITY OF SOUTH CAROLINA
CHARLESTON, SC 29401

WRAY, CHARLIE M, DO
PHYSICIAN
VA SAN DIEGO HEALTH SYSTEMS
ASSOCIATE PROFESSOR OF MEDICINE
UNIVERSITY OF CALIFORNIA, SAN FRANCISCO
SAN FRANCISCO, CA 94121

ZHOU, LI, MD, PHD
PROFESSOR
DEPARTMENT OF MEDICINE
BRIGHAM AND WOMEN'S HOSPITAL
HARVARD MEDICAL SCHOOL
SOMERVILLE, MA 02145

SCIENTIFIC REVIEW OFFICER

HENDERSON, CRYSTAL, EDD
SCIENTIFIC REVIEW OFFICER
OFFICE OF RESEARCH AND DEVELOPMENT
VETERANS HEALTH ADMINISTRATION
DEPARTMENT OF VETERANS AFFAIRS
WASHINGTON, DC 20420

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